Part 2

Is It Morally Acceptable to Buy and Sell Organs for Human Transplantation?

Introduction

"While people's lives continue to be put at risk by the dearth of organs available for transplantation, we must give urgent consideration to any option that may make up the shortfall. A market in organs from living donors is one such option. The market should be ethically supportable, and have built into it, for example, safeguards against wrongful exploitation. This can be accomplished by establishing a single purchaser system within a confined marketplace." The preceding is the abstract from a 2003 paper by Charles Erin and John Harris in the *Journal of Medical Ethics* titled, "An Ethical Market in Human Organs" (Erin & Harris, 2003). Depending upon one's perspective, what they say might strike one person as straightforwardly sensible, or another person as downright reprehensible. Interestingly enough, we praise the person who donates one of their organs, but do not ordinarily praise—and may even condemn—the person who wants to sell a body part. This condemnation occurs notwithstanding the fact that there are many more patients who need organ transplants than there are persons who are willing to provide them altruistically. For example, in 2008, in the United States there were an estimated 139,917 patients waiting for organ donations, yet only 27,281 organ transplants occurred (HRSA, 2009). What might be some of the reasons that a market in human organs is considered reprehensible even in the face of extreme shortage?

Someone of a religious bent might claim that the human body is a sacred temple housing some kind of divine or supernatural entity such as a soul, and *both* body and soul may have been given as a gift from God. Thus, although it is true that the human body is extended in space and time, and subject to physical laws of the universe like any other material thing, its specialness and uniqueness as (a) the temple of the

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soul and (b) a gift from some god(s) means that it should not be treated like any other object, and especially not like a commodity to be bought and sold (May, 1977; CBPC, 1991).

One need not be religious, however, to draw the same conclusion regarding not treating the human body like a commodity. Immanuel Kant's (1724–1804) deontological moral theory emphasizes respect for persons, and this respect can be extended to persons' bodies since, after all, a person can be understood as an *embodied* human being who is rational, free, and responsible (Kant, 1775–89/1963, 1785/1998, 1797/1996; LRCC, 1992; Munzer, 1993; Cohen, 1999; Morelli, 1999).

Interestingly enough, Kant's morality has also been used by proponents of a market in human organs. The part of Kant's morality emphasizing the fact that rational beings are seen as *autonomous*—that is, free to make choices for themselves unimpeded by any coercion—is what becomes significant here. Fundamental rights to privacy and the use of one's body as one sees fit, for example, are viewed as elements of this autonomy, and it is arguable that the right to ownership over one's own body in exercising decisions is the most fundamental of these rights. If an *embodied* human being is autonomous in this sense, then surely she can choose to either donate *or sell* parts of her body, if desired. It is her body, and she can do with her body whatever she sees fit to do with it, including offering it or parts of it to the highest bidder (Markowitz, 1990; Schwarzenbach, 1998; Gill & Sade, 2002; cf. Gerrand, 1999; Merle, 2000; Wilkinson, 2010). Ron Brown (1999) summarizes this commonsense Kantian-based position: "Simply stated, the moral justification is this: each adult owns his body and thus has the absolute right to make all decisions regarding it, providing he abstains from using force or fraud on others."

In the first chapter of this section, Mark Cherry puts forward this "It's my body, and I'll do what I want with it" kind of argument: "A person's authority over himself, his freedom of choice over the use of his body and mind, is part-and-parcel of maintaining personal integrity." This autonomy is oftentimes appealed to in cases where one *donates* an organ, so why shouldn't this autonomy extend to the *selling* of organs? As Cherry notes, "Public policy that prohibits consensual organ procurement from persons who are willing to donate, provided that they receive financial payments or other valuable incentives, denies the very authority of persons over themselves that justifies organ procurement in the first place."

Utilitarian and other consequence-based moral reasons figure as much into the debate concerning the market in human organs, as do Kantian and other deontological-based moral reasons. The standard utilitarian reasoning utilized in favor of a market in human organs is fairly simple: if human organs are needed in the transplant community—which they obviously are—and we can supply them by offering a financial incentive to people, and if people are willing to pay and donors are willing to accept the risks involved, then the end result will be plenty more people surviving, which is an obviously beneficial consequence to the transplant community. Thus, the straightforwardly moral thing to do is allow people to buy and sell human organs, which, at present, is illegal in nearly every nation including the United States (NOTA, 1984; Territo & Matteson, 2012). In the midst of dispelling medical myths, offering counterarguments, and reminding us of the fact that medicine, medical procedures and, in many cases, the human body itself all are instrumental goods in the biomedical marketplace—after all, people sell and pay for a myriad of medical procedures and services, as well as for cadavers, all over the world (Callahan & Wasunna, 2006; Rice & Unruh, 2009)—Cherry also endorses a consequence-based moral reason for a market in human organs,: "altruism-based systems of organ donation are not working adequately, whereas a market in human organs and other body parts would efficiently and effectively save lives, reduce human suffering, encourage highly skilled professional medical practice, and respect the authority of persons over their own bodies."

In the second chapter of this section, Art Caplan notes: "selling organs, even in a tightly regulated market, violates the existing bioethical framework of respect for persons since the sale is clearly being driven by profit." Continuing, Caplan maintains that a market in human organs "violates the ethics of medicine itself" since the "creation of commerce in body parts puts medicine in the position of removing body parts from people solely to abet those people's interest in securing compensation as well as to let

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middle-men profit." Thus, in opposition to Cherry, Caplan is clearly uncomfortable with the fact that medicine and medical procedures exist as marketplace phenomena. But, at least part of this discomfort is well deserved, since money, or the lack thereof, oftentimes determines access to vital medical care (Wong, 1998; Callahan & Wasunna, 2006; Powers & and Faden, 2006; Hansen, 2008). And any market will be rife with exploitation, despite numerous regulations as well as checks and balances (Radin, 1996; Wertheimer, 1996).

In response to the "It's my body, and I'll do what I want with it" kind of argument, Caplan points out the harsh reality that most people who would be willing to sell a body part are poor, so the decision to sell a body part hardly seems rational and free in the robust Kantian sense: "Talk of individual rights and autonomy is hollow if those with no options must 'choose' to sell their organs to purchase life's basic necessities." Especially since those in the medical profession are supposed to uphold the "do no harm" principle, it seems all the more egregious for medical professionals to be engaging in exploitative behavior. In an attempt to sidestep the exploitation that seems to be inherent in any market, at the end of his chapter Caplan favors default to donation programs like those advocated in Spain, Italy, Austria, France, and Belgium. In such a program, an individual is presumed to wish to donate their body or body parts upon death, unless they make the conscious decision to opt out of the program. And Caplan points to data that would seem to indicate that such programs are working.

References

Brown, R. (1999). A free market in human organs. *Freedom Daily*. February 1. Retrieved from: http://www.fff.org/freedom/0296e.asp

Callahan, D., & Wasunna, A. (2006). *Medicine and the market: Equity v. choice*. Baltimore, MD: The Johns Hopkins University Press.

CBPC (Chilean Bishops' Permanent Commission). (1991). On organ transplants. *Catholic International*, *15*(30), 374–375.

Cohen, C. (1999). Selling bits and pieces of humans to make babies. *Journal of Medicine and Philosophy*, *24*(3), 288–306.

Erin, C., & Harris, J. (2003). An ethical market in human organs. *Journal of Medical Ethics*, *29*, 137–138.

Gerrand, N. (1999). The misuse of Kant in the debate about a market for human body parts. *Journal of Applied Philosophy*, *16*(1), 59–67.

Gill, M., & Sade R. (2002). Payment for kidneys: The case for repealing prohibition. *Kennedy Institute of Ethics Journal*, *12*(1), 17–46.

Hansen, F. (2008). A revolution in healthcare: Medicine meets the marketplace. *Institute of Public Affairs (January)*, *43–45*.

HRSA (Health Resources and Services Administration) (2009). 2009 annual report. Retrieved from: http://optn.transplant.hrsa.gov/AR2009.exe

Kant, I. (1775–89/1963). *Lectures on ethics* (L. Infield, Trans.). Indianapolis, IN: Hackett Publishing.

Kant, I. (1785/1998). *Groundwork of the metaphysics of morals* (M. Gregor, Trans.). (Section I: Transition from common rational to philosophic moral cognition). Cambridge: Cambridge University Press.

Kant, I. (1797/1996). *Practical philosophy* (M. Gregor, Trans.). Cambridge: Cambridge University Press.

LRCC (Law Reform Commission of Canada). (1992). *Procurement and transfer of human tissues and organs: Working paper 66*. Ottawa, Canada: Communication Group Publishing.

Markowitz, S. (1990). Abortion and feminism. *Social Theory and Practice*, *16*, 1–17.

May, W. (1977). *Human existence, medicine and ethics*. Chicago: Franciscan Herald Press.

Merle, J. (2000). A Kantian argument for a duty to donate one's organs: A reply to Nicole Gerrand. *Journal of Applied Philosophy*, *17*(1), 93–101.

Morelli, M. (1999). Commerce in organs: A Kantian critique. *Journal of Social Philosophy*, *30*(2), 315–324.

Munzer, S. (1993). Kant and property rights in body parts. *Canadian Journal of Law and Jurisprudence*, *6*(2), 319–331.

NOTA (National Organ Transplant Act: Public Law 98-507). (1984). Retrieved from: http://history.nih.gov/research/downloads/PL98-507.pdf; also see http://optn.transplant.hrsa.gov/policiesAndBylaws/nota.asp

Powers, M., & Faden, R. (2006). *Social justice: The moral foundations of public health and health policy*. Oxford: Oxford University Press.

Radin, M.J. (1996). *Contested commodities: The trouble with trade in sex, children, body parts, and other things*. Cambridge, MA: Harvard University Press.

Rice, T., & Unruh, L. (2009). *The economics of health reconsidered*. Chicago: Health Administration Press.

Schwarzenbach, S. (1998). On owning the body. In J. Elias, V. Bullough, V. Elias, & G. Brewer (Eds.), *Prostitution: On*

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whores, hustlers, and johns (pp. 345–351). Amherst, NY: Prometheus Books.

Territo, L., & Matteson, R. (Eds.). (2012). *The international trafficking of human organs*. Boca Raton, FL: Taylor & Francis.

Wertheimer, A. (1996). *Exploitation*. Princeton, NJ: Princeton University Press.

Wilkinson, M. (2010). Sell organs to save lives. *The British Broadcasting Company (BBC) News*. August 27. Retrieved from: http://www.bbc.co.uk/news/health-10786211

Wong, K. (1998). *Medicine and the marketplace: The moral dimensions of managed care*. Notre Dame, IN: University of Notre Dame Press.

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Chapter Three

It Is Morally Acceptable to Buy and Sell Organs for Human Transplantation

Moral Puzzles and Policy Failures

Mark J. Cherry

In this chapter, I argue that embracing market-based incentives for organ procurement would save lives and reduce human suffering, while helping to contain economic costs and stretch healthcare budgets. Financial and other valuable incentives would encourage living persons to donate redundant internal organs, while also increasing access to nonredundant organs with donors (or their families) granting permission for organ harvesting after death. Legislating "altruism" on the part of donors (or the donor's family), coerces self-sacrifice in an otherwise commercial setting, where surgeons, nurses, pharmaceutical companies, hospitals, third-party procurement agencies, government bureaucrats, as well as organ recipients openly profit. At stake, however, is not solely the efficiency and effectiveness of transplantation practice, but more fundamentally the recognition of the moral authority of persons over themselves. Openly embracing a market in human organs for transplantation would encourage virtuous tendencies in transplantation, shed light on what is often a hazy and shrouded policy setting, while better addressing the shortage in human organs for transplantation.

Introduction

Alicia and James are living kidney donors. Each has arranged to donate a kidney to save the life of a child of a close acquaintance, who is in need of transplant. Both are aware that kidney donation comes with the attendant risks of surgery; each seeks to minimize the risks by working with a well-known hospital with a good reputation for providing high-quality medical care for organ donors. Alicia and James both enjoy the recognition that they are saving a life and reducing human suffering. Many consider their actions to be heroic: saving the life of another at some risk to oneself. While James is donating his redundant kidney for free, Alicia has asked the mother of her organ recipient for a significant cash reward, which the mother has happily agreed to pay. What is the moral difference, if any, between Anna and James?

Paid rescue workers, who risk their lives to help others, are accepted in many facets of life. Fire fighters, police officers, Coast Guard naval rescuers, as well as ski search and rescue teams, each risk life and personal injury to help others while collecting a salary. While one may work as a volunteer fire fighter, that someone

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chooses to be paid for services does not reduce the honor of the occupation. With regard to Alicia and James, the organ retrieval surgery will be exactly the same—indeed, all of the medical aspects of the two cases are identical. Consequently, it is unclear why there ought to be any moral difference between compensated and uncompensated organ donation. The legal difference between the two cases is startling: James's choice is praised, encouraged, the subject of positive educational propaganda, and legal; Alicia's request for compensation is decried, denounced, and subject to legal punishment almost everywhere in the world. Why?

The prevailing moral viewpoint is that human organs may be donated, but they should not be sold, despite the shortfall in organs available for transplantation and the consequent increase in human suffering and the deaths of otherwise salvageable patients. Commentators opine that human organs should not be viewed as commodities to be sold for private gain, but rather treasured as an altruistic gift, a social resource to be distributed according to medical criteria. Prohibitions on financial incentives for organ donation are often couched in lofty moralistic terms (e.g., "preserve the nobility of organ donation" (Declaration of Istanbul); International Summit on Transplant Tourism and Organ Trafficking, 2008, p. 1227) and manipulative educational campaigns (e.g., "Give the Gift of Life"). Organ transplantation, it is claimed, should transcend commercial practices and rise above marketplace morality. Yet, the urgent public health challenge due to the considerable disparity between the number of patients who could benefit from organ transplantation and the number of organs available for transplant will not be adequately addressed by the vilification of commodification and market transactions. Despite the fact that such assumptions underlie nearly all public policy regarding organ procurement and transplantation, I conclude that each is misguided and illegitimate.

Embracing market-based incentives for organ procurement would save lives and reduce human suffering, while helping to contain economic costs and stretch healthcare budgets. Financial and other valuable incentives (e.g., college scholarships, tax incentives) would encourage living persons to donate redundant internal organs (e.g., a kidney), while also increasing access to nonredundant organs (e.g., the heart) with donors or their families granting permission for organ harvesting after death. Legislating "altruism" on the part of donors or the donor's family coerces self-sacrifice in an otherwise commercial setting, where surgeons, nurses, pharmaceutical companies, hospitals, third-party procurement agencies, government bureaucrats, as well as organ recipients openly profit. At stake, however, is not solely the efficiency and effectiveness of transplantation practice, but more fundamentally the recognition of the moral authority of persons over themselves. Openly embracing a market in human organs for transplantation would encourage virtuous tendencies in transplantation, shed light on what is often a hazy and shrouded policy setting, while better addressing the shortage in human organs for transplantation.

Some Background Numbers

To assess current transplantation policy realistically, one must also consider the background risks involved in organ transplantation bereft of the significant and valuable incentives of the commercial market. The United Network for Organ Sharing (UNOS) documents that, in the United States alone, some 7182 patients died in 2008 while waiting for an organ transplant (UNOS, 2009, table 1.6). Many more patients die each year after being removed from the transplant waiting lists because they have become too ill to transplant. Others endure suffering, often in hospital on life support, or in expensive outpatient treatments, while queuing for available organs. In 2008, in the United States only 27,281 organ transplants occurred despite the 139,917 patients who queued on the waiting list at some point during that year. At the end of 2007, the waiting list was populated with 96,874 candidates for transplant, compared to some 100,597 at the end of 2008; UNOS currently lists 111,776 candidates (as of August 4, 2011). There has been a 56.6% increase in demand for transplantable organs from 1999 to 2007 (UNOS, Chapter 1, p. 1). While demand for organs has risen significantly, organ donation rates in the United States have been relatively stagnant for some time, with the availability of organs declining over the past five years (UNOS,

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Chapter 2). Organs are procured primarily from cadaveric sources; however, living donation constitutes a significant portion of kidney donation (5966 out of 16,067), with a growing number of living liver-lobe donations (249 out of 5568) (OPTN, 1999–2004; UNOS, 2009). Altruism-based policies for organ donation have simply not been adequate to meet the medical demand for transplantation.

The usual circumstance of transplant patients without a private donor is an evermore significant wait and risk of death. UNOS reports, for example, that patients in need of a kidney transplant, with blood type O, who registered in 2001/2002, experienced a median wait time of 1833 days; those with blood type B, who registered in 2001/2002, experienced a median wait time of 2033 days. Patients in need of a liver transplant, with blood type O, who registered in 2001/2002, experienced a median wait time of 1228 days (UNOS, 2009). Given increased demand for human organs, and a concurrently increased queuing time, median wait times for patients with less common blood types, and highly sensitized recipients, has become difficult to calculate accurately because fewer patients as a percentage of those queuing have received a transplant since listing (Hippen, 2005; Xue et al., 2005).

As queuing time for organ transplantation has increased, direct and indirect health risks have increased as well. Patients with end-stage renal failure not due to diabetes have a mortality rate of approximately 60% at five years while waiting for organs; morality rates are worse for patients whose renal failure is due to diabetes. Even queuing for less than six months has a long-term negative impact on health risks relative to preemptive transplantation (Meier-Kriesche and Kaplan, 2002; Abou Ayache et al., 2005). Over time, the body becomes more fragile, creating greater likelihood of poor post-transplant outcomes. According to one study, during a 33-month period, 85 transplant candidates on the waiting list at the University of Minnesota died awaiting transplant (63 waiting for kidney transplant, 22 for simultaneous pancreas–kidney transplant). The transplant candidates' mean age at death was 53 (±11) years. Of the 85 patients, 71% were waiting for a first transplant, and 62% had a 0% panel-reactive antibody level at the time of death (see Casingal et al., 2006). As Arthur Matas, Benjamin Hippen, and Sally Satel (2008) comment regarding this study, "it would have been easy to find a kidney for them, if there were a sufficient supply" (p. 380). The median wait time for a donor kidney routinely exceeds the median life expectancy of dialysis-dependent transplant candidates (Matas et al., 2008). If the median wait time for kidney transplantation continues to increase, which seems likely, it will surpass the life expectancy of an evermore significant portion of potential recipients.

Sustaining patients medically while they await transplantation is very expensive. Financial costs to care for the end-stage renal population increased 57% between 1999 and 2004 (Foley & Collins, 2007). Total Medicare costs for end-stage renal disease in 2000 was just over $12 billion; by 2008, Medicare expenditures for end-stage renal disease had increased to over $23 billion. In 2008, the cost per patient for dialysis was $76,587; in comparison, the per-person cost for transplant patients was $26,668, and for patients with a well-functioning graft, the cost was $19,104 (USRDS, 2010, tables K1 K6, K9, and K12). Insofar as wait times for transplantation can be reduced through financial incentives for organ donation, such incentives would save both monetary and medical resources, while decreasing suffering and improving life and health outcomes for patients.

Financial Incentives: Increasing Access to Transplantation

Such suffering is all the more tragic, since much of it could be prevented by legalizing an open market in human organs for transplantation. Creating significant incentives for living-organ donation will multiply the availability of organs, such as kidneys, bone marrow, and liver segments. The financial rewards helped motivate Alicia's decision to donate; it would likely prompt others to do the same. Some persons might be willing to consider a futures contract in which they agree to sell any usable organs upon their death to an organ-procurement agency, and have the money paid as a death benefit to their survivors. Similarly, financial and other valuable incentives would enable families to sell the organs of a decease loved one, rather than just to

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donate the organs. Incentives for families to make available body parts from recently deceased relatives would increase access to nonredundant organs, such as hearts, as well as bones, cornea, and other useful body parts from deceased donors. Knowing that their families would financially benefit would likely encourage many more potential donors to state their intentions to be organ donors.

Financial and other market-based incentives encourage persons to raise resources to further personal goals and social interests. Such incentives drive technological and medical innovation; they possess significant motivational force independent of civic mindedness, self-sacrifice, and social solidarity. Other incentives for organ donation might include organ entitlements (i.e., higher priority on the waiting list for families whose members have donated organs), payment of funeral expenses, life-insurance contracts, and tax credits. If the thought of the donor's mother paying Alicia in cash for her kidney seems unduly crass, money need not change hands. Imagine instead Alicia being provided with a four-year college scholarship, or funding for graduate studies, such as medical or law school—a valuable incentive indeed! It is highly plausible that such valuable incentives would be successful in motivating the availability of human organs for transplantation. Each case is little different from the current system of organ donation except that donors, or their families, receive financial or other valuable compensation to stimulate donation. Utilizing various incentives creatively to fashion public policy could efficiently and effectively increase the availability of organs, thereby improving access to transplantation, reducing human suffering, and saving lives.

Barter markets (trafficking by exchange of commodities) in organs already exist, are morally praised, and are being expanded. Examples include paired kidney exchanges and "triple swap" kidney donation and transplant operations, in which three patients, who are not tissue compatible with their own willing donors, exchange their donor's kidney for a kidney from another of the three donors. Each willing donor provides a kidney to one of the three transplant patients (Delmonico et al., 2004; Saldman et al., 2006). Surgeons at Johns Hopkins University Hospital performed just such an organ swap in 2003, and since then what is often termed "human organ paired donation" has become more commonplace (Ferrari & De Klerk, 2009; Gumber et al. 2011). Kidney exchanges and triple-swap donation programs are forms of reciprocal directed donation. A similar kidney-exchange program was established in the Netherlands (De Klerk et al., 2005, 2006; Mahendran & Veitch, 2007). Importantly, such "directed donations" typically bring organs into the transplant pool that would otherwise not have been available (Paramesh et al., 2011). Nongovernmental programs, such as MatchingDonors.com, help those in need of transplant to arrange for potential organ exchanges. Soliciting donors in this manner increases access to transplantation. Most exchanges have been swaps among kidney donors, but other types of organ trades are possible: for example, a segment of healthy liver could be exchanged for a healthy kidney.

In 2007, section 301 of the United States National Organ Transplant Act (Public Law 110–144) was specifically revised to permit paired kidney exchanges and other types of organ exchanges for transplantation. The original Act prohibited the transfer of human organs for use in transplantation for any sort of "valuable consideration." It prohibited for-profit commercial harvesting, financial incentives, or other valuable consideration to encourage donation or sale of human organs for transplantation. As amended, "human organ paired donation," i.e., kidney swaps and other organ exchanges, do not violate the prohibition on receiving "valuable consideration." Amendment was necessary because paired organ exchanges were becoming increasingly popular, and receiving an organ in exchange for an organ, with each party thereby saving the life of a loved one, is quite obviously the receipt of "valuable consideration." The practice is straightforwardly a "trafficking by exchange of commodities"—a barter market in human organs for transplantation.

Human Organs Are Instrumental Goods

Critics charge that compensating organ donors, or their families, would reduce the human body to no more than an instrumental good, a thing, a commodity

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to be bought and sold. Consider, for example, the comments of S. Daniel Davis and Samuel Crowe (2009):

> Embedded within the ethic of generosity is the idea and the hope that individuals may freely, without any form of coercion, choose to give of themselves for the good—in this case, the health—of another … In deference to the generosity of donors and the health of transplant recipients, physicians may, in good conscience, undertake the necessary surgical and medical interventions, confident that these interventions are consistent with the ends of medicine. If this ethic of generosity, however, gives way to an ethic of buying and selling, then physicians will be willing accomplices to both instrumentalization and commodification—consequences that would harm the profession of medicine. (p. 600)

This criticism, however, is misdirected: it regards not just compensated organ donation but the very practice of organ transplantation itself. One of the key moral issues underlying live organ donation, as highlighted in the examples of Alicia and James mentioned at the very beginning of this chapter, is the recognition that the transplant surgeon harvests a healthy organ from a healthy person so as to benefit some other fully separate person, who is sick and in need of transplant. In the case of deceased donation, which is usually carried out after the determination of death by neurological criteria, judgment is made that the donor no longer needs his organs (see Iltis & Cherry, 2010); consequently, organs can permissibly be surgically removed and placed into some other person. Transplantation demonstrates empirically that organs are manipulable and interchangeable with other organs of the same kind. This is the physical reality that makes transplantation medically viable. The intention is for the harvested organ to cease being a living part of the donor and to become a living part of the recipient, so transplant teams attempt to find a replacement organ that is as similar as possible to the body part they wish to replace. Such considerations underlie the careful histocompatibility testing for tissue-matching donor organs with potential recipients. This testing maps leukocyte antigens on the surface of the body's cells, which help the immune system protect the body against invaders. The immune system also recognizes the histocompatibility antigens from the tissues of other persons and may react against these antigens leading to the rejection of organ grafts. Transplant professionals attempt to match as many of the antigens as possible between the donor organ and the recipient to reduce the change of rejection (UNOS 2009). In short, transplantation medicine relies on the empirical fact that human organs are instrumentally useful things.

All systems of transplantation, including uncompensated organ donation, objectify human organs and utilize them as fungible replaceable objects. As a surgical practice, organ transplantation requires that we appreciate these body parts as exchangeable objects. Organ transplantation demands that we understand body parts as alienable; i.e., that persons can permissibly donate their redundant internal organs to benefit others, and that families can grant permission for organ harvesting after a patient's death. Instrumentalization of body parts does not flow from whether the donor was offered, requested, or received valuable compensation; instrumentalization of body parts is part and parcel of organ transplantation. As the American Medical Association Council on Ethical and Judicial Affairs (1993) noted, organs are a medical resource: "the shortage of organs is the most obvious example of scarcity in medical resources" (p. 1). Donors, surgeons, procurement agencies, and recipients alike objectify organs and treat them as instrumentally useful medical goods.

Indeed, the market in body parts is quite literally a billion-dollar industry, reaching well beyond organ transplantation:

> In the cadaver business, suppliers sell bodies and body parts to brokers, who in turn funnel them to buyers. Suppliers include morgues, medical schools, tissue banks, independent companies, funeral homes, and even, on occasion, crematoria. Brokers, who facilitate the corpse sales, may be independent businessmen or employees in some of the same places. Their clients include medical associations, major US corporations, researchers, doctors, and hospitals. The demand for bodies and parts surpasses the supply, which keeps the prices of human flesh and bones very high. Each corpse that travels through the system can generate anywhere from $10,000 to $100,000, depending on how it is used. (Cheney, 2006, p. 8; see also Waldby & Mitchell, 2006)

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Medical research relies extensively on human body parts. Medical students learn financially valuable information and technical skill while working on human bodies and body parts. Human parts are useful for pharmaceutical, cosmetic, and surgical equipment development, and technical practice, to name but a few marketable uses. Museums display preserved bodies (mummies) as well as the plasticized human bodies in the "Body Worlds" exhibits. The reproduction industry buys and sells sperm and ova (Nelson, 2009), the wig industry buys human hair, and there exists potentially profitable research on human embryos, adult and embryonic stem cells, and human DNA. The market in human plasma, a blood component, is booming with private centers in the United States handling some 18.8 million transactions a year (the current rate is about $20.00 a pint; Kimes, 2009, p. 12). Body parts are commodities.

Marketplace Morality

It is fashionable in bioethics to believe that the role of the market in medicine is less than positive. A quasi-social-democratic political vision and ideological orientation functions as part of the taken-for-granted lingua franca of bioethics. This moral framework seeks to place medicine, and thereby transplantation, above what is characterized as the crass morality of the marketplace. Transplantation is to be set firmly within a morality of generosity and altruism, not the market. As an editorial in the *The Lancet* (2009) opined: "Ethical arguments have been made for and against the practice, with the pro side generally contending that legitimizing a market for organs would increase their availability. But human livers and kidneys are not commodities, and hospitals are not just another convenient locale for money to change hands. Trade in human organs is immoral and ought to be outlawed around the world" (p. 1901).

Having failed to appreciate the ways in which medicine treats livers, kidneys, bone marrow, and so forth as medical resources, the editorial leaves conveniently unstated that surgeons, nurses, hospital administrators, and other staff charge significant amounts of money for access to medical goods and services.

Medicine is a commodity: its goods and services are bought and sold, are valued over against other goods and services, are the subject of economic choices, and are given a monetary equivalence. Hospitals, physicians, and other healthcare workers demand payment for services rendered. Consider these statements regarding payment from the University of Pennsylvania Hospital (2011) website:

> The University of Pennsylvania Health System participates with many medical insurance plans. It is important to understand that your insurance policy is an agreement between you and your insurance carrier. We will submit the claim on your behalf, however that is not a guarantee of payment of the claim. Should your claim be rejected or only partially paid, your insurance company should send you an explanation of benefits. Ultimately, the fees are your responsibility. If we do not participate with your insurance plan, payment is expected at the time of service … Co-pays, deductibles, co-insurances and fees for noncovered services are expected at the time of your service. A $12.00 administrative fee will be assessed if the co-pay is not paid at the time of service. For your convenience we accept cash, personal checks, Visa, MasterCard, American Express, and debit cards.

Even with state-based healthcare systems, citizens purchase medical care through various forms of high taxes. Such financial, billing, and taxing practices straightforwardly place medicine squarely within marketplace morality—that ethic of buying and selling goods and services.

Moreover, there is no good reason to believe that such circumstances are altogether lamentable. Whereas it has become commonplace to remark that medical research and patient care have become a lucrative enterprise, it would be shortsighted not to note the considerable advancements in material well-being and life expectancy that medical innovation has brought to the industrial world, which includes transplantation medicine (see Cherry, 2006). For-profit medical research has played a crucial role in demonstrating that many treatments once thought to be important parts of healthcare are ineffective, or at least less effective than new alternatives (consider, e.g., the development of advanced immunosuppresives to prevent organ graft rejection; see Ponticelli, 2011). Innovation has increased longevity while decreasing

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morbidity. Within transplantation, the market has led to important pharmaceutical innovation and greater use of "expanded criteria donors" or "marginal donors"; that is, "donors that would not have been considered suitable for donation previously" (Humar, 2004; UNOS, 2009). Such organs include those that in the past were believed to have an unacceptable risk for primary nonfunction or initial poor function. The positive result of such expanded criteria is that more organs are available for transplantation; the negative aspect is that such transplants often have inferior results, such as greater rates of complications or graft rejection. For liver transplantation, for example, there is a clear correlation between post-transplant outcomes and the quality of the organ graft (Mittler et al., 2008; Müllhaupt et al., 2008). Medical innovation requires the significant investment of economic resources. Financial rewards for organ donation would create significant incentives for innovation in many such areas.

Some critics believe that financially compensating organ donors involves an exchange of incommensurable values. The concern is that financial compensation will fail appropriately to weigh and compare economic versus noneconomic values. However, nonmarket-based strategies for procurement and allocation face similar difficulties. Government-based organ-confiscation policies, for example, are often framed to appear altruistic, even when they are in fact coercive. Presumed consent, for example, involves no actual consent from any actual person; instead, organs are simply taken, unless the individual has specifically and officially made his rejection of organ donation known, such as by signing the properly official forms. Similar challenges beset proposals for "routine retrieval" policies. Imagine a policy of "presumed consent to one's house": unless one specifically and officially stated otherwise, the government coercively takes ownership of one's house upon one's death. Even if it served some plausible state use, such a policy would be neither altruistic nor consensual. Estate taxes are confiscatory nonaltruistic contributions to the state. The government simply takes the funds for its own purposes. Normally, unless otherwise specified in a will or other legal document, the deceased's property—house or bodily remains—passes to the ownership of her/his heirs.

Financial transactions and other types of market exchanges do not require that the goods exchanged be precisely commensurable; such a restriction would rule out nearly all consensual transactions. Permissible transactions require that the parties transact voluntarily, that deception or other forms of coercion are not employed, and that each party agrees to the value or product to be received. This means that what is received in return is worth at least as much to the party as that which was given at the time of the transaction. One can buy or sell "priceless" works of art without claiming that its aesthetic or historic value is commensurate with the money that is paid (Wertheimer, 1992, p. 218). A "priceless" Picasso—"Nude, Green Leaves, and Bust"—was sold in May 2010, at auction in Christie's New York for some US$106.5 million. Money equivalence is usually understood in terms of what persons are willing to pay for the transfer of ownership, even in the case of so-called "priceless" objects.

Similar equivalences can be created for organs and other body parts. As a practical matter, if necessary, it should be possible to establish minimum prices with enforceable contractual duties for medical follow-up, so that compensation will be sufficient to have a significant positive impact on the donor's (or the donor's family's) life. For example, the valuable compensation could be framed to include education, vocational, or other training, on the model of the G.I. Bill, which provides tuition benefits for the college education of those who have served in the US armed forces. Although limiting compensation for organ donation to college scholarships seems to be unduly restrictive, they would be, as I have argued elsewhere, at least a step in the right direction (Cherry, 2009b).

Coerced Altruism

The persistent reference to organ donation as an "altruistic gift" framed within an "ethic of generosity" may be more lofty rhetoric than empirical reality. Presumably, many organ donations are fully motivated by altruistic concerns; however, it is difficult to know what percentage falls squarely into this category without any private negotiation for side-benefits, or implicit understandings of a quid pro quo. Human

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motivations are complex, multifaceted, and not always fully acknowledged. Requiring strict adherence to altruism may cause more harm than benefit. Most living donations are to family members. These donations are plausibly motivated by love or beneficence; however, they are just as plausibly motivated by family loyalty, gratitude, guilt, or avoidance of the shame of failing to donate. If James is the only member of his family who is tissue-compatible to donate a kidney to his niece, there will be a great deal of social pressure on him to donate—especially if the alternative is the niece's continued suffering and death. For these donors, their willingness to donate stems from their relationship with the particular patient and may not be fully altruistically motivated. Perhaps by donating a kidney to his niece, something he has good reason to do anyway, James believes that his uncle will reciprocate by paying for his college education or by providing a generous inheritance. Transplant surgeons also have complex motivations for practicing their profession: desires to do good, to obtain technical skill, to be paid a high salary, to gain reputation and professional recognition, each of which is compatible with providing high-quality surgical care. It holds donors to too high a standard to presume that they ought to be motivated only by an ethic of generosity or altruism. James should be permitted to save his niece's life, even if his motivations are in part self-serving.

Parents have put forth minor children as living donors, with parents consenting on behalf of those children. In *Hart v. Brown* 289 A.2d 386 (Conn. Super. Ct 1972), the court ruled that the parents of twin girls could consent on behalf of one twin to have a kidney procured and transplanted into her sister. As Angela Holder (1990) documents, "Almost all courts have granted these orders, based on the idea that the donor child derives benefit from the continued life of the sick sibling. The courts' theory is that a child derives more benefit from a happy home and the sibling relationship that he or she would from growing up with two kidneys. Other courts have dispensed with this concept of benefit and have simply found that parents are allowed to consent to donation by one healthy child to a sick child on the basis of a familial cost–benefit analysis" (p. 523). Couples have utilized in vitro fertilization with genetic testing so as to select embryos for implantation that, once born, are destined to become tissue donors for an already-living sibling (Sheldon & Wilkinson, 2004). The donor's altruism in such cases is nonexistent; nor is it plausible that the parents involved are acting selflessly through an ethic of generosity. Still, parents, together with transplantation professionals, seem better placed to make such medical and moral decisions than government bureaucrats.

That transplant recipients, surgeons, insurance companies, and organ procurement agencies would prefer to obtain human organs without financially compensating donors is understandable, but this fact does not imply that potential organ donors act wrongly when they seek or require valuable compensation prior to agreeing to donate. Access to organ transplantation is not free: transplant surgeons, hospitals, and organ procurement agencies charge a great deal of money for their services and expertise; pharmaceuticals and access to other medical goods and services are very expensive. Yet, the only party who is legally prohibited from seeking or accepting compensation for his part in this lucrative market is the organ donor (or his family). Forbidding compensated organ donation legislates altruism and self-sacrifice in an otherwise commercial setting. As long as transplantation utilizing human organs continues, these body parts will be appreciated as highly valuable scarce medical resources. What is at stake is not whether organs should be treated as commodities, but rather who should receive the valuable medical resource and who should bear the costs of appropriation and transfer.

That human organs can only be transferred at a price of zero does not reduce the value of the organs to zero. It straightforwardly transfers the value of the organ from the donor to other parties. Consequently, financially compensating donors of organs and other body parts would also be significantly fairer than the current system of prohibition. Insofar as legal statutes prohibit donors from accepting financial compensation, human organs are a highly constrained commodity, where the state requires donors to part with their valuable property without material compensation, whereas others benefit financially, and the recipient of the organ benefits physically, as well as perhaps financially, in terms of being able to return to work, reduced medical bills, and increased quality and quantity of life.

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Persons and Their Bodies

An additional conceptual puzzle concerns the authority of persons to grant permission to organ procurement surgery—Alicia and James are each consenting to the same medical procedure. Common law jurisprudence has appreciated a right of persons to be secure against battery, a right not to be touched, which was grounded in the authority of persons over themselves, rather than in any particular view of the best interests of persons. The weight of this moral and legal tradition establishes persons as in authority to make their own judgments regarding acceptable risks and benefits as they collaborate with others through freedoms of association and contract. Persons are appreciated as possessing a dignity that should not be violated by being touched or used without permission, but who may consent to more or less risky activities. Persons may grant permission to be used in ways that, absent their consent, would be profoundly harmful. Permission marks the distinction, for example, of rape versus free love, sexual assault versus welcome seduction, assault, and battery versus kidney removal surgery. Persons may consent to risky activities and lifestyle choices—joining the military, working on oilrigs, engaging in tattooing and body piercings, breast enhancement or other forms of plastic surgery, or participating in promiscuous or risky sex. Individuals routinely set life and health at risk for national patriotism, for career advancement, or to enhance one's attractiveness to potential sex partners. Others need not approve of such choices; they may even decry the consequences as imprudent or deeply unfortunate. However, there is a prima facie lack of moral authority to interfere in the choices of persons who freely choose to act with consenting others.

Despite rhetoric to the contrary, human organs are not a scarce public resource; organs are retrieved from private persons, who have presumptive authority over themselves, their bodies, and the uses to which parts of their bodies will be put. A person's authority over himself, his freedom of choice over the use of his body and mind, is part and parcel of maintaining personal integrity. Absent agreement regarding the demands of God, or the requirements of moral rationality, individuals have been identified within rather broad side-constraints as the best judges of their own best interests and of their own preferred method for attempting to realize such interests. The authority of persons over themselves is core to the respect of persons that lies at the heart of moral and legal reflections on informed consent to medical treatments, which appreciates persons as able to grant permission to (even risky) medical activities—for example, by choosing to donate a kidney while living, or granting permission for one's organs to be donated upon one's death. The very jurisprudential tradition that makes organ donation morally permissible—the authority of persons over themselves, their own bodies and minds—is denied in the case of compensated organ donation, even though all of the medical aspects of the two cases are identical. Public policy that prohibits consensual organ procurement from persons who are willing to donate, provided that they receive financial payments or other valuable incentives, denies the very authority of persons over themselves that justifies organ procurement in the first place.

Moreover, such policy demeans persons, seeking to supplant personal judgment regarding one's own best interests with the judgment of government bureaucrats and academic bioethicists. It is the presumption that one knows what is in the best interests of others, what is most appropriate for their well-being, without even having asked the persons themselves. Such moralistic substituted judgment combined with legal prohibition on compensated donation uses other persons without their permission to achieve a particular view of moral propriety and human dignity. It fails to respect persons as capable of making prudential and moral decisions about their own fates. It denies persons the opportunity to choose on the basis of their own judgments how best to advantage themselves.

Conclusion

The development of a market for the procurement of organs provides no reason to stop asking patients or their families to consider donation. In the United States, extensive charity infrastructures exist side by side with for-profit markets for food, medicine, and housing. Financial incentives do not preclude the liberties of the altruistically inclined to realize their

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need to take care of others. Unless governmentally prohibited, even within a market system, private individuals could still donate organs out of charity to family members or to others in need. Social and political institutions that support the free choices of persons to interact with free and consenting others are neutral with regard to the expression of charity; market-based liberties include, but are not limited to, profit-seeking interests. Presuming that the willingness to donate body parts is motivated by actual, rather than coerced, altruism, those who are willing to donate should still be willing to donate regardless of the existence of a market. Additional strategies designed to increase organ availability, such as directed donation, should not be seen as exclusive alternatives to the market. Pursuing multiple parallel strategies may lead to the greatest organ availability. It may be, however, as I have argued in more detail elsewhere, that the goals of increasing organ availability, controlling medical costs, and reducing human suffering would be more effectively and honestly secured with the existence of an open market in human organs for transplantation (see Cherry, 2005).

Market incentives encourage persons to raise resources to further personal as well as social interests and goals. Profits from organ sales would allow for the private pursuit of business and educational opportunities, or to further more public agendas. Given that social and personal advantage is often tied to education and business success, such incentives may be significant. Commercialization would create opportunities, which many may view as attractive, to secure resources for pursuing their own educational, business, political, and welfare interests. It is likely that utilizing the market as a procurement strategy would encourage individuals, who would not otherwise donate, to sell their organs, which would increase the availability of organs for transplantation. The market is also very likely to increase access to nonredundant organs and body parts with harvesting authorized by the families of deceased donors. An open market (unlike the black market) would discourage unscrupulous practices and can be tracked and regulated. Successful procurement and transplantation require the skilled services of many professionals. Hospitals and highly skilled transplant surgeons have significant professional incentives to encourage virtuous tendencies in the medical marketplace. In additional to specific contractual obligations, surgeons and other medical personnel would be governed by the usual professional medical standards of practice. Such standards are nearly impossible to enforce on the current black market (see, e.g., Stacey Taylor, 2009).

Treating uncompensated donation and compensated donation as if such practices were inherently different is morally implausible. Prohibiting compensated organ donation forbids competent adults from engaging in a commercial transaction from which both parties expect to benefit. One may not approve of the choices of other persons, but they are the ones in authority to make such decisions regarding their own lives and bodies. The goal of informed consent to medical treatment, for example, is not simply to endorse patient autonomy as a positive value, but to respect personal autonomy as a side constraint—an acknowledgment that the burden of proof on others, including governments, to interfere in the free choices of persons regarding their bodies is indeed significant. In short, altruism-based systems of organ donation are not working adequately, whereas a market in human organs and other body parts would efficiently and effectively save lives, reduce human suffering, encourage highly skilled professional medical practice, and respect the authority of persons over their own bodies. With an open market, the availability of organs would not be limited to acts of altruism, government coercion, or manipulative educational campaigns designed to convince people to donate their organs for free. Failing to acknowledge that human organs are commodities, even while public policy, commercial interests, and transplantation medicine treats them as such, encourages continuation of a dishonest social political fiction. Prohibiting compensated donation denies persons the opportunity to choose freely on the basis of their own judgments regarding how best to advantage themselves, while condemning those waiting for available organs to significant suffering and increased risk of death.

Acknowledgment

Distantly ancestral versions of some of these arguments appeared in Cherry (2009a).

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References

Abou Ayache, R., Bridoux, F., Pessione, F., Thierry, A., Belmouaz, M., Leroy, F. ... Touchard, G. (2005). Preemptive renal transplantation in adults. *Transplantation Proceedings*, *37*(6), 2817–2818.

American Medical Association Council on Ethical and Judicial Affairs. (1993). *Ethical considerations in the allocation of organs and other scarce medical resources among patients*. CEJA Report K-A-93. Retrieved from: http://www.ama-assn.org/resources/doc/code-medical-ethics/216a.pdf

Casingal, V., Glumac, E., Tan, M., Sturdevant, M., Nguyen, T., & Matas, A. (2006). Death on the kidney waiting list—good candidates or not? *American Journal of Transplantation*, *6*, 1953–1956.

Cheney, A. (2006). *Body brokers: Inside America's underground trade in human remains*. New York: Broadway Books.

Cherry, M. J. (2005). *Kidney for sale by owner: Human organs, transplantation and the market*. Washington, DC: Georgetown University Press.

Cherry, M. J. (2006). Medical innovation, collapsing goods, and the moral centrality of the free-market. *The Journal of Value Inquiry*, *40*(2–3), 209–226.

Cherry, M. J. (2009a). Why should we compensate organ donors when we can continue to take organs for free? A response to some of my critics. *The Journal of Medicine and Philosophy*, *34*, 649–673.

Cherry, M. J. (2009b). Embracing the commodification of human organs: Transplantation and the freedom to sell body parts. *Saint Louis University Journal of Health Law & Policy*, *2*, 359–377.

Davis, F. D., & Crowe, S. J. (2009). Organ markets and the ends of medicine. *The Journal of Medicine and Philosophy*, *34*(6), 586–605.

De Klerk, M., Keizer, K., Claas, F., Witvliet, M., Haase-Kramwijk, B., & Weimar, W. (2005). The Dutch national living donor kidney exchange program. *American Journal of Transplantation*, *5*(9), 2302–2305.

De Klerk, M., Witvliet, M., Haase-Kramwijk, B., Claas, F., & Weimar, W. (2006). A highly efficient living donor kidney exchange program for both blood type and crossmatch incompatible donor–recipient combinations. *Transplantation*, *82*(12), 1616–1620.

Delmonico, F., Morrissey, P., Lipkowitz, G., Stoff, J., Himmelfarb, J., Harmon, W. ... Rohrer, A. (2004). Donor kidney exchanges. *American Journal of Transplantation*, *4*(10), 1553–1554.

Ferrari, P., & De Klerk, M. (2009). Paired kidney donations to expand the living donor pool. *Journal of Nephrology*, *22*(6), 699–707.

Foley, R., & Collins, A. (2007). End-stage renal disease in the United States: An update from the United States renal data system. *Journal of the American Society of Nephrology*, *18*, 2644–2648.

Gumber, M., Kute, V., Goplani, K., Shah, P., Patel, H., Vanikar A. ... Trivedi, H. (2011). Transplantation with kidney paired donation to increase the donor pool: a single-center experience. *Transplantation Proceedings*, *43*(5), 1412–1414.

Hippen, B. (2005). In defense of a regulated market in kidneys from living vendors. *The Journal of Medicine and Philosophy*, *30*(6), 627–642.

Holder, A. (1990). Legal issues in bone marrow transplantation. *The Yale Journal of Biology and Medicine*, *63*, 521–525.

Humar, A. (2004). Maximizing the donor pool: Marginal donors, splits, and living donor liver transplants. *Journal of Gastroenterology and Hepatology*, *19*, S410–S413.

Iltis, A., & Cherry, M. J. (2010). Revisiting death and the dead donor rule. *The Journal of Medicine and Philosophy*, *35*(3), 223–241.

International Summit on Transplant Tourism and Organ Trafficking. (2008). Declaration of Istanbul on organ trafficking and tourism. *Clinical Journal of the American Society of Nephrology*, *3*, 1227–1231.

The Lancet. (2009). Editorial: Legal and illegal organ donation. *The Lancet*, *369*(9577), 1901.

Kimes, M. (2009). Blood money. *Fortune*, *159*(12), 12.

Matas, A., Hippen, B., & Satel, S. (2008). In defense of a regulated system of compensation for living donation. *Current Opinion in Organ Transplantation*, *13*, 379–385.

OPTN (Organ Procurement and Transplantation Network). (1999–2004). All Kaplan–Meier median waiting times for registrations listed: 1999–2004. Retrieved from: http://optn.transplant.hrsa.gov/latestData/rptStrat.asp

Paramesh, A., Hanley, K., Slakey, D., Killackey, M., Zhang, R., & Buell, J. (2011). Who's your donor? Bringing about Louisiana's first domino paired exchange transplants. *Journal of the Louisiana State Medical Societ*, *163*(2), 102–104.

Ponticelli, C. (2011). Present and future of immunosuppressive therapy in kidney transplantation. *Transplantation Proceedings*, *43*(6), 2439–2440.

Mahendran, A., & Veitch, P. (2007). Paired exchange programmes can expand the live kidney donor pool. *British Journal of Surgery*, *94*(6), 657–664.

Meier-Kriesche, H., & Kaplan, B. (2002). Waiting time on dialysis as the strongest modifiable risk factor for renal transplant outcomes: a paired donor kidney analysis. *Transplantation*, *74*(10), 1377–1381.

Mittler, J., Pascher, A., Neuhaus, P., & Pratschke, J. (2008). The utility of extended criteria donor organs in severely ill liver transplant recipients. *Transplantation*, *8*, 895–896.

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Müllhaupt, B., Dimitroulis, J., Gerlach, P., & Clavien, A. (2008). Hot topics in liver transplantation: organ allocation—extended criteria donors—living donor liver transplantation. *Journal of Hepatology*, *48*(1), S58–S67.

Nelson, L. (2009). New York State allows payment for egg donations for research. *The New York Times*, June 26. Retrieved from: http://www.nytimes.com/2009/06/26/nyregion/26stemcell.html

Sheldon, S., & Wilkinson, S. (2004). Should selecting saviour siblings be banned? *Journal of Medical Ethics*, *20*, 533–537.

Saldman, S., Roth, A., Sönmez, T., Unver, M., & Delmonico, F. (2006). Increasing the opportunity of live kidney donation by matching for two- and three-way exchanges. *Transplantation*, *81*(5), 773–782.

Stacey Taylor, J. (2009). Autonomy and organ sales, revisited. *Journal of Medicine and Philosophy*, *34*, 632–648.

University of Pennsylvania Hospital. (2011). Retrieved from: http://www.pennmedicine.org/pat_ins/

UNOS (United Network for Organ Sharing). (2009). The 2009 annual report of the OPTN and SRTR: transplant data 1999–2008. Retrieved from: http://www.unos.org/donation/index.php?topic=data

USRDS (United States Renal Data System). (2010). USRDS 2010 annual data report: Atlas of chronic kidney disease and end-stage renal disease in the United States. Retrieved from: http://usrds.org/reference.htm Retrieved from: http://www.usrds.org/reference.htm

Waldby, C., & Mitchell, R. (2006). *Tissue economies*. Durham, NC: Duke University Press.

Xue, J., Ma, J., Louis, T., & Collins, A. (2005). Forecast of the number of patients with end-stage renal disease in the United States to the year 2010. *Journal of the American Society of Nephrology*, *16*(5), 2753–2758.

Wertheimer, A. (1992). Two questions about surrogacy and exploitation. *Philosophy and Public Affairs*, *21*, 211–239.

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Chapter Four

It Is Not Morally Acceptable to Buy and Sell Organs for Human Transplantation

A Very Poor Solution to a Very Pressing Problem

Arthur L. Caplan

Proposals to increase the supply of organs must be very carefully weighed against the prevailing ethical framework of voluntary altruism. Those values are widely known and heavily promoted, and have long served to protect the interests of prospective donors. Changes in these values might well alienate the public or healthcare professionals who have grown used to this ethical framework. Major religious groups who support this bioethical framework or healthcare workers, the majority of whom believe that the current bioethical framework is the appropriate one to govern organ procurement, could react very negatively to any major shift in the ethical infrastructure of organ and tissue procurement. Shifting to markets not only risks alienating important segments of the community who support altruism but also makes it difficult to condemn the practice of trafficking in persons and parts for transplant. And a market does nothing to help increase the supply of hearts and livers while making it less likely that these will be donated while those selling their kidneys get paid. The other key ethical problem facing a shift to some form of market is that it asks physicians to do harm to otherwise-healthy patients solely so that they may personally profit by being made sicker through removal of an organ.

The Harsh Reality of Allowing Markets in Organs—Trafficking of the Poor

Levy Izhak Rosenbaum, an Orthodox Rabbi in Brooklyn, New York liked to refer to himself as a "matchmaker." However, he was not arranging dates for his congregants. Rosenbaum was one of five rabbis indicted on July 23, 2009 in New Jersey for brokering the sale of black-market kidneys and, in a few cases, lobes of livers. He is accused of finding poor, Syrian Jews, who spoke little English or Hebrew and were newly immigrated to Israel, and paying them $10,000 to travel to the US to sell a kidney to patients waiting in various US transplant centers.

Rosenbaum was quite a businessman. He pocketed as much as $150,000 per organ from purchasers for serving as the middleman in his organ trafficking scheme. He not only short-changed his organ sources but even charged them for their transportation and room and board (Sherman & Margolin, 2011).

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Rosenbaum knew that buying organs was illegal. So, he was sure to always describe payment to his kidney sources as compensation for their time. Nevertheless, his scheme constituted blatant trafficking in persons for the purpose of obtaining organs.

Those involved were compelled by poverty to think about selling one of their kidneys. A middleman then took advantage of their poverty and illiteracy, duping them into thinking that they would earn a large sum of money free and clear by selling a kidney. Sadly, in most of the world, allowing financial incentives in kidneys means that trafficking, exploitation, disabling the sellers and even killing is the reality (Rothman & Rothman, 2006a, 2006b; Budiani-Saberi & Delmonico, 2008; Foster, 2009).

Trafficking is an all-too-real phenomenon in the world of kidney transplantation. It can take many forms. Sometimes, people are brought to hospitals against their will or with no true informed consent to "sell" a kidney. Sometimes, would-be patients travel to hospitals where they will meet someone, found by a broker or middleman, who, having been paid, will make a kidney available. And, in some instances, the organs themselves are simply forcefully removed, sometimes after an execution in a prison, and then sent, for a fee, to waiting patients, some of whom come from other countries, at nearby hospitals (Sify News, 2011).

Properly preserved kidneys remain viable for transplant for many hours. They can be flown thousands of miles for the right price. Studies estimate that nearly 10% of all kidneys transplanted around the world are trafficked (WHO, 2007; Caplan et al., 2009).

The New Jersey indictments represent the first known instance of trafficked human organs reaching patients in the United States. But, as numerous reports have documented, there are many examples of organ markets that are nothing more than trafficking occurring in many locations around the globe (COE, 1997; Caplan et al., 2009). In the past few years, wealthy persons needing transplants have traveled from the United Arab Emirates to Sri Lanka, from the United States to Azerbaijan, and from many nations around the world to Pakistan, Egypt, China, and Iraq to receive kidneys sold by desperately poor, ill-informed, uneducated persons. The sellers of these organs are only of interest to those trafficking them as sources of income. Once a kidney is removed, the seller's fate in terms of follow-up care and transportation home is of no concern at all to traffickers.

A typical case of selling a kidney involved a 64-year-old American who underwent kidney transplantation in Baku, Azerbaijan at the end of May 2009. The seller was a 31-year-old poor Ukrainian previously unknown to the recipient. Individuals outside of Azerbaijan arranged the transplant, but it was performed in Azerbaijan by an Israeli transplant surgeon. After the transplant, the seller was left to fend for himself including finding the means to return home. The details of this incident were reported to the Israeli and Azeri authorities by the physician who cared for the patient upon his return to the United States post-surgery (Postrel, 2011).

Patients from the United Arab Emirates, Kuwait, Oman, Australia, the Netherlands, Turkey, Kosovo, and India have undergone transplantation at the Kidney Center of Rawalpindi and the Aadil Hospital of Lahore, Pakistan. Trafficked organs obtained from very poor, often illiterate individuals with little capacity for true informed consent and usually deeply in debt were used (Jawaid Attari, 2009).

Not all trafficking involves crossing national borders. The television network, Al Jazeera, reported on July 20, 2009 that organ brokers were arranging for poor Iraqis to sell their kidneys at the Al-Khakal hospital in Baghdad (Al Jazeera, 2009). And, according to numerous press accounts, despite government condemnation, there continues to be a brisk market in organs taken from the very poor in India and from prisoners in China (Fearon, 2007).

Sales can lead to behavior that goes beyond simply taking advantage of poverty to obtain a kidney. The Associated Press reported on July 20, 2009 that a Saudi Arabian man married a Filipino woman as a cover for buying her kidney. This phony marriage was concocted to circumvent a recent law prohibiting foreigners from undergoing transplantation in the Philippines (Mail Online, 2009).

There are few defenders of trafficking as a solution to organ shortage. Forcing, coercing, or duping adults into making an organ available or removing a kidney from a child or a nonconsenting prisoner, alive or dead, is inconsistent with international human rights conventions but also violates the ethical norms of the

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professional transplant community (SCIS, 2008). Trafficked persons, those who are duped, forced, coerced, or deceived, are being exploited, even if they are paid.

Trafficking has been condemned by all international bodies that have examined the trade (COE, 1997; WMAGA, 2006; SCIS, 2008; Caplan et al., 2009). Trafficking is a reprehensible response to scarcity. So, why does trafficking persist? And is it possible to create systems using financial incentives to obtain kidneys that would not degenerate into trafficking? If not financial incentives, then are there other ideas or strategies that might alleviate the shortage of transplantable kidneys and other organs?

Scarcity—Bad, Underestimated, and Growing Worse

Every day, dozens of people die around the world while waiting for transplants. Many more await bone, cornea, dural matter, tendon, and other tissue transplants, suffering severe disability while they wait. These deaths and lives struggling with disabilities are especially tragic, since many might be prevented if more organs and tissues were available for transplantation. Scarcity means that hard choices have to be made about who will live and who will die. With more than 100,000 people on waiting lists for kidneys, hearts, livers, lungs, and intestines in North America alone, the pressure to find organs is enormous.

Scarcity, however, is growing worse every year. Waiting lists are growing faster than the supply of organs. Physicians are becoming more adept at dealing with harder cases in performing successful transplants (Reese et al., 2010). Aging populations in many nations increase the demand for kidneys. And increases in the rates of obesity, diabetes, and hypertension are driving up the demand for more kidney transplants as well. The capacity to perform transplants is spreading to many nations around the globe, increasing the demand for organs worldwide (WMA, 2011).

Scarcity is actually a worse problem than it appears to be from published data on demand. Demand for transplants is actually underestimated from that shown on public waiting lists in Canada, the US, Europe, Asia, the Middle East, and South America. If there were greater access to primary healthcare, more people might be identified as needing an organ or tissue transplant before becoming too sick to survive a transplant. And if transplant centers were to relax their current admissions standards to include more people—such as the those who lack money or insurance, those who have severe intellectual disabilities, older persons, prisoners, and foreigners who cannot get transplants in their own countries due to a lack of transplant centers and surgeons—then the lists of those waiting in rich nations could easily triple or quadruple (Quinn et al., 2007).

Duties to Those in Need and Duties to Those Who Might Supply an Organ

Those who care for persons dying of renal failure know the terrible toll the shortage of transplantable kidneys takes. Some doctors and nurses are, apparently, willing to remain ignorant of the provenance of the organs they transplant out of the belief that their sole ethical duty is to their dying patients. Transplant teams do, however, have an ethical duty to protect donor interests. Intentional ignorance about how an organ was obtained—from a scheduled execution of a "prisoner" or an organ that has been trafficked—is not an acceptable ethical excuse on the part of transplant teams and hospitals (COE, 1997; Caplan et al., 2009). Need creates moral pressure, but respect for the dignity, autonomy, and health of living donors is important as well, particularly since the integrity of donor procurement systems relies upon the fulfillment of these duties to maintain trust of all involved. Transplant teams are quite simply obligated to protect the interests and health of the sources of organs as well as recipients.

Ultimately, transplant teams and hospitals must be held accountable for knowing the origin and source, or provenance, of the kidneys and any other organs they transplant (Caplan et al., 2009). Trafficking and execution on demand flourish, not because anyone has made a convincing case they are morally or legally defensible but, in part, due to the willful ignorance of transplant centers eager to help their patients or transplant tourists while turning a profit.

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In considering the merits of proposals for inducing people to sell organs, which basically amounts to a strategy that is only applicable to kidneys, it is necessary to understand fully the bioethical framework that has guided organ and tissue donation since solid organ transplants' inception in the 1950s (Caplan & Coelho, 1998).

The Prevailing Ethical Framework for Obtaining Organs and Tissues

The existing bioethical framework for obtaining organs and tissues in most parts of the world and exemplified in professional society norms (SCIS, 2008) is grounded on four key values—respect for persons, the autonomy of the individual, voluntary consent, and altruism. The notion that organs or tissues can be removed from a body for the purposes of transplantation, living or dead, without voluntary consent has not been accepted, except in highly unusual circumstances (i.e., unclaimed bodies at morgues under a coroner's jurisdiction; see Bagheri, 2005; Indiana State, 2009). Persons and their families are recognized in law and ethics as having a controlling interest over the disposition of the body upon death. Deceased and living persons are to be treated with dignity and not merely used to serve the needs of others.

Even though someone might well benefit from obtaining my liver or having bone marrow from my body, these organs and tissues ought not be removed from me, whether I am alive or newly dead, without my permission. To remove them after my death, if I have expressed no preference, then the consent of a surrogate (e.g., a family member, partner, or guardian) must be sought. To do otherwise is to commit a battery or assault upon a living person and to desecrate the body of a newly deceased person. The act of treating persons with dignity is exemplified by affording them control over the disposition of their body and its parts in life and upon death (Santiago, 1997; IOM, 2006). Respect is also shown by following the wishes of the newly dead when they were alive.

Another core element of the existing ethical framework governing the procurement of organs and tissues is that the body and its parts not be made the object of commerce. Prohibitions against slavery and trafficking of persons for prostitution are based upon the bioethical principle that respect for the inherent dignity of human beings requires that they not be bought and sold (Lincoln, 1863; UNGA, 1948; COE, 1997; Caplan et al., 2009). The transplant community has incorporated this view into a prohibition against trading in body parts for profit. In part, this position reflects the fundamental dignity of persons that is exemplified by prohibiting their being enslaved, bought, or sold. The emphasis on altruism in the existing values framework, as reflected in the use of the term *donation*, signals the notion of human dignity is respected by putting sale off limits while permitting gifts (Cohen, 2002).

In order to obtain organs and tissues from the living, there is broad agreement that, ethically, one must only seek a kidney from a competent person who is fully informed and who can make a voluntary, uncoerced choice. In the situation where organs and tissues are sought from the recently deceased, the notion of voluntary consent has been extended, in many nations, to the recognition of donor cards as adequate to direct a donation post-mortem with registration of intent to donate while alive in computerized registries. Variants of policies about who bears the duty to consent exist, but voluntary, informed consent is crucial in making organ and tissue procurement ethical (Sperling, 2009).

Proposals to increase the supply of organs must be very carefully weighed against this prevailing ethical framework, since it is widely known, heavily promoted, and has long served to protect the interests of prospective donors. Changes in these values might well alienate the public or healthcare professionals who have grown used to this ethical framework. Major religious groups who support this bioethical framework or healthcare workers, the majority of whom believe that the current bioethical framework is the appropriate one to govern organ procurement, could react very negatively to any major shift in the ethical infrastructure of organ and tissue procurement (Henegan, 2008).

Increasing the Supply

A number of steps have been taken over the years in many nations to try to increase the supply of organs. An early effort in the 1970s was to enact laws permitting

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the use of organ donor cards that allowed family consent to donate a deceased relative's organs. Some nations began requiring hospitals to ask all patients' families about organ and tissue donation upon death—so-called required request laws (Caplan, 1986, 1988). Some countries require hospitals to honor a patient's donor card, even when a family member opposes donation.

These policies were somewhat effective in gaining more organs from cadaver donors but the gap between supply and demand continued to increase. Therefore, some now argue for a shift away from a reliance on voluntary altruism in organ donation toward a paid market or system that uses financial incentives (Satel, 2008, 2009; Halpern et al., 2010).

Two basic strategies have been proposed to provide incentives for people to sell their organs upon their death. One strategy is simply to permit organ sale by allowing persons to broker contracts while alive with persons interested in selling at prices mutually agreed upon by both parties (Radcliffe-Richards et al., 1998; Satel, 2006; Taylor, 2009). Markets already exist on the Internet between potential live sellers and people in need of organs (Caplan, 2004; Barclay, 2004).

The other strategy is a "regulated" market in which the government would act as the purchaser of organs—setting a fixed price and enforcing conditions of sale (Harris & Erin, 2002; Matas, 2004; Gimbel & Strosberg, 2010). Iran appears to have such a market in operation, although reports on how it is actually being implemented and how well it functions in terms of protecting sellers are not encouraging (Griffin, 2007). Both proposals have drawn deserved ethical criticism.

The Trouble with Markets in Kidneys

One criticism is that only the poor and desperate will want to sell their body parts. If you need money, you might sell your kidney to try and feed your family or to pay back a debt. This may be a "rational" decision, but that does not make it a matter of free, voluntary choice (Hughes, 2006; Caplan et al., 2007).

Watching your child go hungry when you have no job, and a wealthy person waves a wad of bills in your face, is not exactly a scenario that inspires confidence in the "choice" made by those with few options except to sell vital body parts. Talk of individual rights and autonomy is hollow if those with no options must "choose" to sell their organs to purchase life's basic necessities. Choice requires information, options, and some degree of freedom, as well as the ability to reason about risks without being blinded by the prospect of short-term gain (Feinberg, 1986; Beauchamp & Childress, 1979/2009).

It is hard to imagine many people in wealthy countries eager to sell their organs either while alive or upon their death. In fact, even if compensation is relatively high, few will agree to sell (Rid et al., 2009). That has been the experience with markets in human eggs for research purposes and with paid surrogacy in the United States—prices have escalated, but there are still relatively few sellers (Baylis & McLeod, 2007).

Moreover, markets, if successful, would only have an impact on kidneys. It is not clear what the impact would be of creating a market in kidneys from living persons on the rest of the altruistically oriented organ procurement system. The risk of alienating altruistic donors of hearts, lungs, faces, limbs, and livers by creating a narrow market in kidneys is a huge one to take, given the potential loss of life involved for others. Selling organs, even in a tightly regulated market, violates the existing bioethical framework of respect for persons, since the sale is clearly being driven by profit. It also violates, in the case of living persons, the ethics of medicine itself. The core ethical norm of the medical profession is the principle, "Do no harm." The only way that removing an organ from someone seems morally defensible is if the donor chooses to undergo the harm of surgery solely to help another not to make money. The creation of commerce in body parts puts medicine in the position of removing body parts from people solely to abet those people's interest in securing compensation as well as to let middle-men profit (Rothman & Rothman, 2006a, 2006b).

Is this a role that the health professions can ethically countenance? In a market—even a regulated one—doctors and nurses still would be using their skills to help living people harm themselves solely for money. In a cadaver market, they would risk making families and patients uncertain about the

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degree to which appropriate care was being offered and continued if a person might be deemed by healthcare workers or greedy relatives worth more "dead than alive." The resulting distrust and loss of professional standards is a high price to pay to gamble on the hope that a market may secure more organs and tissues for those in need (Harmon & Delmonico, 2006).

A Better Option—Default to Donation

There is another option for increasing the organ supply that has been tried in Spain, Italy, Austria, France, and Belgium. These nations have enacted laws that create a presumption that individuals wish to donate their organs. Following public opinion polls that show majoritarian support for organ donation, instead of asking people to opt in to the donation of their organs the laws in these nations ask those who do not wish to donate to opt out. In such a system, the presumption is that a deceased person wants to be an organ donor upon their death—basically an ethical default to the desirability of donation (Caplan, 1983, 1994). People who do not want to be organ donors can say so while alive by carrying a card indicating their objection, or by registering their objection in a computerized registry, or by doing both. They may also tell their loved ones and rely on them to object should procurement present itself as an opportunity (BMA, 2010). Any close relative could also act to prohibit donation if they believed that was not the desire of the deceased. A more felicitous description of presumed consent is *default to donation*. Individuals are familiar with such defaults in a way that might make them far more comfortable with this strategy for obtaining more organs (Cass & Thaler, 2009).

What is remarkable about this strategy is that it has worked! Unlike the hypothetical and unsupported assertions of advocates of financial incentives and markets that there will be interest in sale in rich nations or the ability to control trafficking in poor ones, default to donation has produced results. This policy has done so without creating any problems or difficulties in the nations that have enacted such legislation. And it has done so without any significant increase in cost beyond the costs of education, training, and public-health campaigns about the new policy (Abadie & Gay, 2006; Gil-Diaz, 2009; Verheijde et al., 2009).

What is important about this strategy from a bioethical perspective is that it is completely consistent with the existing, longstanding bioethical framework of voluntary altruism governing organ and tissue procurement. Respect for persons and voluntary, altruistic consent remain the moral foundation for making organs available.

Conclusion

The worldwide shortage of transplantable organs has led to a significant degree of trafficking in kidneys. Some of the groups involved in this illicit trafficking are also linked to trafficking in women and children for prostitution. These markets are immoral on their face. Worse, it is unlikely that permitting trade in kidneys could ever be subject to the kind of oversight and policing that would prevent outright trafficking. Given the well-documented horrors of trafficking, there is no plausible case for shifting to markets or regulated markets in poor nations, especially since this removes any incentive nations lacking cadaver donor programs might have to do so and only contributes, possibly, to the supply of kidneys.

Shortage has led some to call for policies that would legitimize the sale of kidneys. But it is difficult to believe that in most parts of the world, sufficient oversight and government authority exist to regulate markets and that they would not quickly deteriorate into trafficking. Even in nations that might be able to regulate a market in kidneys, it is not evident that this would lead to an increase in the overall supply. There is little empirical evidence that money is a key factor in guiding decisions about making kidneys available for transplant. Nor is it likely that any but the most desperately poor or disadvantaged would be drawn to kidney sale, making a mockery of the entire notion of autonomous, free choice to sell a body part. Markets would also create an untenable situation for healthcare workers asking them to use their skills to harm patients solely for the purpose of allowing them a one-time chance to earn money.

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There are alternatives to creating markets. Greater efforts can be made to secure all organs including kidneys by enacting presumed consent legislation. This has been done in some nations with notable results. With sufficient training and educational resources, default to donation policies is known to boost the supply of all organ for transplantation. And by outlawing financial systems, those nations that have made little serious effort to build procurement systems using cadaver organs will be led to do so, thereby decreasing the pressures that contribute to exploitative and immoral organ trafficking.

In the long run, bioengineering organs is the answer to shortage (Marcchiarini et al., 2008). In the short run, protecting the viability of the existing system for obtaining organs by not drastically deviating from the core values that have long prescribed what can be done is the best strategy for helping those in need.

References

Abadie, A., & Gay, S. (2006). The impact of presumed consent legislation on cadaveric organ donation: A cross-country study. *Journal of Health Economics*, *25*(4), 599–620.

Al Jazeera, (2009). Poverty drives Iraq organ trade. *English Aljazeera*. July 20. Retrieved from: http://english.aljazeera.net/news/middleeast/2009/07/200972052636416787.html

Bagheri, A. (2005). Organ transplantation laws in Asian countries: A comparative study. *Transplantation Proceedings*, *37*, 4159–4162.

Barclay, L. (2004). Organ donation via Internet raises ethical concerns: An expert interview with Arthur L. Caplan, Ph.D. *Medscape.com*. October 22. Retrieved from: http://www.med scape.com/viewarticle/491837

Baylis, F., & McLeod, C. (2007). The stem cell debate continues: The buying and selling of eggs for research. *Journal of Medical Ethics*, *33*(12), 726–731.

Beauchamp, T., & Childress, J. (1979/2009). *Principles of biomedical ethics*. Oxford: Oxford University Press.

BMA (British Medical Association). (2010). Organ transplantation and donation. Retrieved from: http://www.bma.org.uk/ethics/organ_transplantation_donation/organdonation brief.jsp.

Budiani-Saberi, D., & Delmonico, F. (2008). Organ trafficking and transplant tourism: A commentary on the global realities. *American Journal of Transplantation*, *8*, 925–928.

Caplan, A. L. (1983). Organ transplants: The costs of success. *The Hastings Center Report 13*(6), 23–32.

Caplan, A. L. (1986). Requests, gifts, and obligations: The ethics of organ procurement. *Transplantation Proceedings 18*(3), 49–56.

Caplan, A. L. (1988). Professional arrogance and public misunderstanding. *The Hastings Center Report*, *18*(2), 34–37.

Caplan, A. L. (1994). Current ethical issues in organ procurement and transplantation. *The Journal of the American Medical Association*, *272*(21), 1708–1709.

Caplan, A. L. (2004). Organs.com: New commercially brokered organ transfers raise questions. *The Hastings Center Report*, *34*(6), 8.

Caplan, A. L., & Coelho, D. H. (Eds.). (1998). *The ethics of organ transplants: The current debate*. Amherst, NY: Prometheus.

Caplan, A. L., Tan, H., Marcos, A., & Shapiro, R. (Eds.). (2007). *Living Organ Transplantation*. New York: Informa Healthcare USA, Inc.

Caplan, A. L., Dominguez-Gil, B., Matesanz, R., & Prior, C. (2009). Trafficking organs, tissues and cells and trafficking in human beings for the purpose of the removal of organs: Joint Council of Europe/United Nations study. Retrieved from: http://www.coe.int/t/dghl/monitoring/trafficking/docs/news/OrganTrafficking_study.pdf

Cass, R. S., & Thaler, R. H. (2009). *Nudge*. New York: Penguin Group.

COE (Council of Europe). (1997). Convention for the protection of human rights and dignity of the human being with regard to the application of biology and medicine. Retrieved from: http://conventions.coe.int/Treaty/en/Treaties/html/164.htm

Cohen, C. B. (2002). Public policy and the sale of human organs. *Kennedy Institute of Ethics Journal*, *12*(1), 47–64.

Fearon, P. (2007). Kidney sales brisk on India's black market. *Newser.com*. May 9. Retrieved from: http://www.newser.com/story/2038/kidney-sales-brisk-on-indias-black-market.html

Feinberg, G. (1986). *Harm to self: The moral limits of the criminal law*. New York: Oxford University Press.

Foster, P. (2009). China admits organs removed from prisoners for transplants. *The Telegraph*. August 26. Retrieved from: http://www.telegraph.co.uk/news/worldnews/asia/china/6094228/China-admits-organs-removed-from-prisoners-for-transplants.html

Gil-Diaz, C. (2009). Spain's record organ donations: Mining moral conviction. *Cambridge Quarterly of Healthcare Ethics*, *18*(3), 256–261.

Gimbel, R. W., & Strosberg, M. A. (2010). Kidney donation: When all else fails, try a regulated market. *Journal of the National Medical Association (January)*. Retrieved from:

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http://findarticles.com/p/articles/mi_7640/is_201001/ai_n49423324/

Griffin, A. (2007). Kidneys on demand. *British Medical Journal*, *334*, 502–505.

Halpern, S., Raz, A., Kohn, R., Rey, M., Asch, D., & Reese, P. (2010). Regulated payments for living kidney donation: An empirical assessment of the ethical concerns. *Annals of Internal Medicine*, *152*, 358–365.

Harmon, W., & Delmonico, F. (2006). Payment for kidneys: A government-regulated system is not ethically achievable. *Clinical Journal of the American Society of Nephrology*, *1*(6), 1146–1147.

Harris, J., & Erin, C. (2002). An ethically defensible market in organs: A single buyer like the NHS is an answer. *British Medical Journal*, *325*, 114–115.

Henegan, T. (2008). Pope slams human organ trade, warns on transplants. *Reuters.com*. November 7. Retrieved from: http://www.reuters.com/article/idUSTRE4A658820081107

Hughes, P. (2006) Ambivalence, autonomy, and organ sales. *Southern Journal of Philosophy*, *44*, 237–251.

Indiana State. (2009). Indiana state law code IC 36-2-14-19 2009. Retrieved from: http://www.in.gov/legislative/ic/code/title36/ar2/ch14.html

IOM (Institute of Medicine). (2006). *Organ donation: opportunities for action*. Washington, DC: The National Academies Press.

Jawaid Attari, M. (2009). Mr. Shahbaz Sharif, please act. *Urdunews*. June 17. Retrieved from: http://urdunews.wordpress.com/2009/06/17/mr-shahbaz-sharif-please-act/

Lincoln, A. (1863). The emancipation proclamation. *Archives.gov*. Retrieved from: http://www.archives.gov/exhibits/featured_documents/emancipation_proclamation/

Mail Online. (2009). Saudi man marries Filipino as cover to buy one of her kidneys. *Mail Online*. July 21. Retrieved from: http://www.dailymail.co.uk/news/article-1200926/Saudi-man-marries-Filipino-cover-buy-kidneys.html

Marcchiarini, P., Jungebluth, P., Tetsiuhiko, G., Asmaghi A., & Rees, L. (2008). Clinical transplantation of a tissue-engineered airway. *The Lancet*, *372*, 2023–2030.

Matas, A. J. (2004). The case for living kidney sales: Rationale, objections and concerns. *American Journal of Transplantation*, *4*, 2007–2017.

Postrel, V. (2011). … With functioning kidneys for all. *The Atlantic*. June 8. Retrieved from: http://www.theatlantic.com/doc/200907u/kidney-donation

Quinn, R. R., Manns, B. J., & McLaughlin, K. M. (2007). Restricting cadaveric kidney transplantation based on age: the impact on efficiency and equity. *Transplantation Proceedings*, *39*(5), 1362–1367.

Radcliffe-Richards, J., Daar, A., Guttmann, R., Hoffenberg, R., Kennedy, I., Lock, M. … Tilney, N. (1998). The case for allowing kidney sales. *The Lancet*, *351*, 1950–1952.

Reese P., Abt, P., Bloom, R., Karlawish, J., & Caplan, A. L. (2010). How should we use age to ration health care? Lessons from the case of kidney transplantation. *Journal of the American Geriatrics Society*, *58*(10), 1–7.

Rid, A., Bachmann, L., Wettstein, V., & Biller-Andorno, N. (2009). Would you sell a kidney in a regulated market system? Results of an exploratory study. *Journal of Medical Ethics*, *35*(9), 558–564.

Rothman, D. J., & Rothman, S. M. (2006a). *Trust is not enough: Bringing human rights to medicine*. New York: New York Review Books.

Rothman, D. J., & Rothman, S. M. (2006b). The hidden cost of organ sale. *American Journal of Transplantation*, *6*, 1524.

Santiago, C. (1997). Asking for the family consent: analysis and refusals. *Transplantation Proceedings*, *29*, 1629–1630.

Satel, S. (2006). The kindness of strangers and the cruelty of some medical ethicists. *The Weekly Standard*. May 29. Retrieved from: http://www.weeklystandard.com/Content/Public/Articles/000/000/012/249jxzdt.asp

Satel, S. (2008). *When altruism isn't enough: The case for compensating kidney donors*. Washington, DC: AEI Press.

Satel, S. (2009). The case for paying organ donors. *The Wall Street Journal*. October 18. Retrieved from: http://online.wsj.com/article/SB10001424052748704322004574477840120222788.html

SCIS (Steering Committee of the Istanbul Summit). (2008). Declaration of Istanbul on organ trafficking and transplant tourism and commercialism. *The Lancet*, *372*(9632), 372–373.

Sherman, T., & Margolin, J. (2011). *The Jersey sting: A true story of crooked pols, money-laundering rabbis, black market kidneys and the informant who brought it all down*. New York: St. Martin's Press.

Sify News. (2011). Oz kidney buyers giving China's "transplant tourism" a boost. *Sify News*. February 6. Retrieved from: http://www.sify.com/news/oz-kidney-buyers-giving-china-s-transplant-tourism-a-boost-news-international-lcgpOcbgcgj.html

Sperling, D. (2009). Israel's new brain-respiratory death act. *Reviews in Neurosciences*, *20*(3–4), 299–306.

Taylor, J. S. (2009). Autonomy and organ sales, revisited. *Journal of Medicine & Philosophy*, *34*, 632–648.

UNGA (United Nations General Assembly). (1948). The universal declaration of human rights. *Un.org*. Retrieved from: http://www.un.org/en/documents/udhr/

Verheijde, J., Rady, M., McGregor, J., & Friederich-Murray, J. (2009). Enforcement of presumed-consent policy and

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willingness to donate organs as identified in the European Union Survey: The role of legislation in reinforcing ideology in pluralistic societies. *Health Policy*, *90*(1), 26–31.

WHO (World Health Organization). (2007). WHO proposes global agenda on transplantation. March 30. Retrieved from: http://www.who.int/mediacentre/news/releases/2007/pr12/en/index.html

WMA (World Medical Association). (2011). WMA statement on human organ donation and transplantation. Retrieved from: http://www.wma.net/en/30publications/10policies/t7/

WMAGA (World Medical Association's General Assembly). (2006). Statement on human organ donation and transplantation. Retrieved from: http://www.wma.net/en/30publications/10policies/t7/

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Reply to Caplan

Mark J. Cherry

Art Caplan's evocative critique of market-based incentives for increasing access to organ transplantation, while rhetorically engaging, routinely misses the mark. At key junctures, his argument either fails adequately to distinguish between legal and illegal markets or relies on ambiguous and moralistic terminology, such as human dignity and personal autonomy. Perhaps most puzzling, Caplan endorses nonconsensual state expropriation of human organs for transplantation from the deceased. A chilling thought, indeed!

Compensated and uncompensated organ donation are not morally distinct. Provided that the procedure takes place in a suitably sterile environment, with adequate surgical skill and attention to the health of the donor, all medical aspects of the two practices are identical. The only distinction is that money changes hands, which, in itself, is not intrinsically morally problematic. For example, surgeons routinely insist on payment for performing life-saving transplant surgery. Caplan rightly notes that illegal markets are open to abuse. However, unlike horror stories surrounding black markets in human organs, the open market would discourage unscrupulous practices. Long-term successful transplantation programs require the skilled services of many professionals. Hospitals, highly skilled transplant surgeons, and their dedicated surgical teams have significant professional incentives to encourage virtuous tendencies in the medical marketplace. In addition to contractual obligations, surgeons, other medical personnel, and institutions would be subject to the usual professional medical standards of practice, and liable to both tort and criminal law for breaches of contract, negligence, or malpractice. Such standards are nearly impossible to track, much less enforce, on the black market. Here, the moral challenge is represented not by financial compensation per se, but by shortsighted public policy.

Defense of compensated organ donation need not affirm personal autonomy as a value, much less as an over-ridding value. Rather, autonomy ought to be appreciated as a side-constraint on the permissible behavior of others. Morally authorized action generally requires, except in emergency and exigent circumstances, that persons not be touched or used without their permission. Moreover, persons, not government bureaucrats or bioethicists, are the presumptive judges of their own best interests (Cherry & Engelhardt, 2004). Persons are morally in authority over themselves. This moral and jurisprudential understanding is core to the practice of informed consent in medicine, including informed consent to organ donation.

Here, the conceptual puzzle is that Caplan both affirms and denies this moral authority of persons over themselves—it is affirmed to justify uncompensated organ donation, but denied to condemn compensated organ donation. This contradiction lies at the heart of contemporary transplantation policy.

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Caplan's intuition that financial compensation violates "human dignity" must be further specified and critically defended. Not all agree on the meaning or implications of human dignity; nor is it clear why such dignity would be so fragile as to be harmed by compensated organ donation. In part, one must adjudicate among moral intuitions, distinguishing between justified and unjustified claims to the loss of human dignity, prior to presupposing that such claims ought to play any role in public policy. Many have deep-seated views, for example, regarding the significant violation of human dignity represented by abortion and common, presumably consensual but high-risk, sex acts. Yet, these are practices that society permits and, indeed, in many quarters celebrates. One might, for example, profitably compare the likelihood of personal harm represented by routinely engaging in high-risk sex acts—e.g., sexually transmitted diseases, virally mediated diseases, such as HIV and HPV-related cancers, as well as long-term risks of infertility, especially among women—with the low risk of one-time, living-kidney donation.

More generally, highly paternalistic regulation that treats adult citizens as mere children, unable to make competent judgments in their own best interests, is hardly affirming of one's dignity. One may not approve of the choices of other persons (e.g., engaging in high-risk sex acts, or accepting financial incentives to donate an organ), and individuals ought to bear the full costs of their choices, but they are the ones in authority to make such decisions regarding their own lives and bodies.

Presumed consent, which Caplan endorses, raises further and difficult challenges. First, *presumed consent* is a mere rhetorical euphemism—a legal fiction, as it were. No actual consent has taken place. If the state simply appropriated all of one's wealth upon death, rather than recognizing that one's worldly goods rightly become the property of one's legitimate heirs, unless one specifically opted out, it would be implausible to conclude that such confiscation was consensual. The hue and cry that such state-based theft would engender is easy to anticipate. Presumed consent is inconsistent with altruism-based transplantation policy.

Second, it is unclear how presumed consent would ally Caplan's expressed concerns that "Deceased and living persons are be to be treated with dignity and not merely used to serve the needs of others." Presumed consent policies are specifically designed to instrumentalize and objectify the human body, to take human organs to serve the needs of others. Such policy manipulates the language of "consent" to exploit citizens, treating them as a mere means to the end of organ appropriation to benefit others.

In summary, as I have argued elsewhere (Cherry, 2005), I do not doubt the sincerity or good intentions of those of my colleagues who support the continued prohibition of compensated organ donation. En passant, commentators in favor of financial incentives for organ donation were not invited to participate in the conference that led to the *Declaration of Istanbul*. It is not difficult to issue a consensus statement on the supposed existence of "human rights," when you only invite activists who are already known to agree with the preferred ideological position that the conference organizers expect to endorse. Such circumstances straightforwardly call into question the supposed universality of the *Declaration*'s assertions regarding such human rights.

I continue to be perplexed, however, by the ways in which such prohibitions exploit vulnerable populations to support highly paternalistic views of moral propriety, human dignity, and inappropriate use of one's body, thereby denying persons the opportunity to choose freely on the basis of one's own judgments how best to advantage oneself, as well as condemning those who are waiting for the availability of organs for transplant to continued suffering and increased risk of death.

References

Cherry, M. J. (2005). *Kidney for sale by owner: Human organs, transplantation and the market*. Washington, DC: Georgetown University Press.

Cherry, M. J., & Engelhardt, H. T. (2004). Informed consent in Texas: Theory and practice. *The Journal of Medicine and Philosophy*, *29*(2), 237–252.

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Reply to Cherry

Arthur L. Caplan

Professor Cherry has made a strong case for permitting markets in kidneys. However, he has not made a persuasive one. Three reasons stand out as to why this is so.

First, Professor Cherry presents us with the idealized hypothetical of Anna and James both making a kidney available to a child—James from altruism, Anna for money. The problem with the case example is that there are likely to be very few Annas selling organs for cash, "aware that kidney donation comes with the attendant risks of surgery … [seeking] to minimize the risks by working with a well-known hospital with a good reputation for providing high-quality medical care for organ donors." Many of the Annas of the real world are likely to be at their wits' end about how to find money to pay overwhelming debts. They are likely to be unemployed. They are likely to be poorly educated. They are likely to have mental- and physical-health issues of their own. In other words, many who might wish to sell will not really be making a *choice*. They will be acting solely out of desperation when they perceive no other options available to them.

The market Professor Cherry idealizes is not selling one's sperm, hair, plasma, or even eggs. It is one requiring undergoing major surgery with real risk of death or disability for cash. Some Annas may meet Cherry's description and thoughtfully choose to sell a kidney. Others—and I would suggest a good number—will be told by their bookie, abusive boyfriend or husband, or loan shark to find a way to come up with some short-term cash and, having no options and thus no real choices, sell a kidney.

Even worse, many Annas will live in other nations who will emulate our decision to permit markets. Those Annas will have even less potential for choice and will simply be coerced, bullied, threatened, or forced into kidney sales. When a market opens in the United States, it also opens in far less lawful and far more impoverished parts of the globe. Since our ability to combat trafficking for organs, sex, baby sales, and indentured slave labor depends on the moral position that incentives in these domains are wrong, it is a bitter price to pay to allow a few Annas in the US to sell what will be forced from many, many more in other parts of the world.

Second, Cherry argues that medicine is a business: "Medicine is a commodity: its goods and services are bought and sold, valued over against other goods and services, are the subject of economic choices, and are given a monetary equivalence. Hospitals, physicians, and other healthcare workers demand payment for services rendered." Therefore, he concludes, we can have doctors paid and patients paid to undergo surgery to take out their organs for no reason other than profits.

Medicine is a business, but it is also a profession—one that relies on trust. If commercial concerns are seen as overwhelming the protection of patient

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interests, then medicine will not long be able to function. If doctors do useless tests on patients solely to make money, then patients come to distrust recommendations for tests. If doctors will remove your kidney, cornea, lobe of liver, or limbs solely so that you and they may turn a buck, patients soon will come to completely distrust their doctors. Transplantation depends upon trust—to obtain organs such as hearts and lungs, people must believe their loved ones are truly dead before removal. Trust in that the surgeon will not give you an inferior or infected organ just to get a paycheck. Trust in that you cannot bribe your way to access to an organ ahead of those in greater need. There is nothing that will destroy trust more in transplant than showing that doctors are quite willing to harm their patients—especially those who are poor or vulnerable—solely and only for money.

Lastly, Cherry believes that markets will work. They will generate more kidneys to transplant. He bases his case on the fact that markets have relieved scarcity in other areas of life. But, markets have also driven up scarcity—a lack of jobs comes to mind in the US as markets ship jobs overseas or permit downsizing in the name of short-term profit. Markets have not done much to improve the credit situation of Americans. Nor have they proven their merit in distributing access to minimal healthcare to all Americans.

There is every reason to believe that markets will not produce a gain in kidneys. Major religious groups such as the Catholic Church vigorously oppose sales in body parts (see, for example, http://www.catholicnewsagency.com/news/pope_condemns_organ_transplant_abuses_as_abominable/). If major religious organizations condemn markets and proscribe participation in any system that tolerates them, then not only will there not be an increase in kidney availability, but also there could well be a drop in the availability of kidneys and of all other organs and tissues used in transplantation. Introducing known and quite zealous major religious opposition to markets in body parts into the realm of transplantation is a far more concrete reason to predict their failure than any of the generalities about markets that Cherry offers in predicting their success.

In the short run, there are other policy strategies, such as default to donation and presumed consent, that truly protect individual choice, which merit trying without turning to markets. In the long run, the solution to shortage in organs is through encouraging more government support for research on artificial organs, xenografting, and stem-cell regeneration. There will be enormous markets for the products that these strategies, if vigorously pursued, will eventually create.

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